

EMREE

Past-Paper Intelligence File

Recalls 2012 – 2025 • 120 Original EMREE-Style Questions • Separated by Subject

BRUTAL • HONEST • PRACTICAL — What actually exists, where to find it, and high-yield practice built from real recall patterns

120 QUESTIONS • 7 SUBJECTS • BLUEPRINT-MATCHED

IM 24 • OBG 18 • Peds 18 • Surgery 18 • Family+Ethics 18 • Psych 12 • Public Health 12 = 120

October 2026 • Evidence cut-off: 4 Oct 2026 • Organiser: National Institute for Health Specialties (NIHS), UAE University • CBT: 120 SBAQs in 3 hours, no negative marking

Honest warning on page 1: NIHS does not publish official past papers. Any PDF sold as 'official EMREE past paper' is a memory-recall reconstruction (students writing what they remember) or a commercial question bank. This file does not leak real exam items. It gives you (1) a verified map of where recalls actually live, (2) the repeat topics examiners love, and (3) 120 brand-new practice questions written from those patterns so you can train safely and legally.

How to use this file in 3 passes: 1) Read pages 2–4 once (exam facts + sources + repeat list). 2) Do 20 questions timed (90 sec each), mark answers. 3) Read explanations only for wrong/guessed items and log the topic. Repeat. Aim for $\geq 75\%$ on mocks – the pass mark moves (55–60% by standard-setting).

WHAT'S INSIDE

- 1 EMREE in one page – organiser, format, blueprint, pass mark, dates, fee ... p.2
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A Internal Medicine (24) B Obs & Gynae (18) C Paediatrics (18) D Surgery (18) E Family Medicine + Ethics (18) F Psychiatry (12) G Public Health / Biostats (12)

- 5 Answer grid for self-marking + 10-week plan + ruthless checklist ... end

1 EMREE IN ONE PAGE — NO FLUFF

EMREE = Emirates Medical Residency Entrance Examination (code EMREE120). Run by the National Institute for Health Specialties (NIHS) at UAE University, Al Ain. It is an **entrance exam for residency training**, not a licence to practise (licensing is DHA / DOH / MOHAP via Prometric/Pearson). Confusing the two costs you a whole cycle.

Item	Verified position (Oct 2026)
Paper	120 single-best-answer MCQs, 4–5 options, computer-based, 3 hours (~90 sec/Q), no negative marking – never leave blank.
Blueprint (NIHS 7-domain)	Internal Medicine 20% (~24) • Obs&Gynae 15% (~18) • Paediatrics 15% (~18) • Surgery 15% (~18) • Family Medicine 15% (~18) • Psychiatry 10% (~12) • Public Health/Preventive 10% (~12). Condensed 5-domain version merges Family into others (IM+EM 40%, OBG 20%, Peds 15%, Surgery 15%, Psych/Ethics/Prev 10%).

Competencies	Diagnosis & investigations 25–30% • Drug therapy 15–20% • Non-drug management 15–20% • History/exam 10–20% • Basic science 10–20% • Prevention 5–10% • Professionalism/ethics 5–10%. Translation: half the paper is 'what is it + what next'.
Pass mark	Set by standard-setting, moves by cycle. 55% quoted by banks, 60% published for 2025–26 medical cycle. Target 75%+ on mocks; a bare pass ranks poorly for seats.
When / where	Season Dec–Mar each year, several sittings (e.g. Dec 2025–Mar 2026 had 10 sittings). Venue: UAE (Al Ain). First-come first-served, seats cap early. Results ~4–10 days.
Fee / attempts / validity	AED 1,000 per attempt ex-tax, non-refundable. Up to 3 attempts per academic year (some guides say 4 – confirm on registration letter). No re-sit same year after pass. Score valid 2 years.
Who can sit	MBBS/MD + final-year students. UAE nationals + IMGs. IMGs often cite <4-year gap rule + visit visa needed. English: IELTS ≥6 / TOEFL ≥80 / OET C+ for residency application, plus ECE evaluation if foreign graduate.

Ruthless tip: two subjects decide you. Internal Medicine + Obs/Gynae ≈ 35–60% depending which blueprint table you use. Candidates who revise 'curriculum order' instead of 'blueprint order' fail. Do IM first, then OBG/Peds/Surgery, then bank Psychiatry + Public Health – the cheapest 20% of marks.

2 WHERE THE REAL 'PAST PAPERS' LIVE

Direct answer: there is **no** official NIHS past-paper PDF. What circulates are (a) old EMSTREX samples (EMREE's former name), (b) student recall docs on Scribd/Drive, (c) commercial banks that rephrase recalls. Links change; search the exact titles below if a link dies. Paywalls are noted so you don't waste money blindly.

Source	What it really is	Access
1. EMSTREX 2012 Sample (NIHS-era sample)	Google Drive folder 'Emree 2022–2023 cycle questions' contains 'SAMPLE QUESTIONS FOR EMSTREX 2012.pdf' + 'EMREEBlueprint 2.pdf'. Best free starting point for style.	Free (Drive)
2. Blog archive 2008 / 2011 / Jan-2014 / Feb-2014	emreeexam.home.blog/past-papers – lists 4 downloads (2008-exam, 2011-mcq, 04-Jan-2014, 15-Feb-2014). Links now mostly dead/outdated – useful only to prove age of repeats.	Free but stale
3. Scribd '00-EMREE 2004–2020' archive	Scribd doc 784545742 – compiles 2011–2020 recalls by subject. Heavy repeats. Requires Scribd subscription/download.	Paywall
4. Scribd 'EMREE Q-BANK Edited Part 1' (to 22 Feb 2020)	Scribd doc 556790674 – manual with all recalls till Feb 2020 categorised by subject + study plan. Most-cited source.	Paywall
5. Scribd 'EMREE 5 MARCH' (2024 cycle, ~112 Q)	Scribd doc 641712306 – 'NEW QUESTIONS IN RED'. Rich in ethics/stats/derm/OBG/peds recalls used to build this file (cohort omega-3, sensitivity 75%, near-miss, psoriasis 10%, Hirschsprung, NPH, anorexia BMI 16.5, lithium, etc.).	Paywall
6. Scribd 'Emree 2 Mar 2025' recalls	Scribd doc 889408204 – 2025 memory cases across peds/psych/cards. Confirms rise in psych + biostats + ethics.	Paywall
7. Drive 'Emree 2022–2023 cycle questions'	drive.google.com/drive/folders/16aZqgMaBCQgFAXgGg-WkNOVR3J-zjRVU – candidate-shared cycle file. Access varies; request access.	Free / request
8. Dr Irin's Academy – Past Paper 289 Q	dririnsacademy.com .../past-paper-for-emree-residency-exam – 289-Q bank + 4000+ bank split by cardiology/neuro/endo/nephro/pulmo/gastro/haem/onc/ID/psych/rheum/derm/ophtho/genetics/pharm/epi/ethics/surgery/ENT/ortho/vascular/peds/OBG.	Paid course
9. ExamCure – Real Recalls 2023–25 + free 25-Q PDF	examcure.com – free 25 EMREE-style Qs (diagnostic) + paid 200+ recall booster (Dec 2023, Jan 2025, Mar 2025) + 4000-Q bank + 50-Q free mock. Explicitly states samples are educational, not official items.	Free sample / paid bank
10. EmreePrep / First Medical / Prometric Medical / DW-SME guides	emreeprep.com, firstmedicalconsultancy.com, prometricmedical.com, drwasimsme.com – free blueprint, dates, demo Qs (e.g. ectopic, torsion, pyloric stenosis, Rh HDN, placenta praevia). Use for blueprint + technique, not as past papers.	Free guides

How to use them without getting burned: (1) Never memorise letters – stems repeat, options shuffle. Learn the principle. (2) Verify every recall answer with a guideline (NICE/ACOG/WHO/CDC) – recall docs contain errors and the authors say so. (3) If a file asks for

3 WHAT REPEATS — 40+ PATTERNS DISTILLED

These are not invented. Each line below traces to at least one recall file above (phrasing normalised, answers verified). If you only have 2 weeks, master this table before any textbook.

Domain	Repeat cues to memorise
Medicine / Emergency	Post-op PE day 10 pleuritic pain; pre-op EF 65% → no echo; asymptomatic chest pain/syncope → stress test; sulfonylurea failure HbA1c 9% → insulin; GLP-1 for weight loss; Hashimoto in pregnancy → increase thyroxine; DI labs (low urine SG, high serum osmo, hyperNa); contact dermatitis (belt rash); psoriasis 10% + Koebner → topical combo; venous ulcer → compression; syncope on nitrates/alpha-blocker → orthostasis.
Obs / Gynae	Ectopic (amenorrhoea + bleed + cervical excitation); torsion (sudden + vomit + mass); praevia (painless + transverse); rubella triad → pre-conception vaccine; amnio 15–16w definitive; giardia in pregnancy → paromomycin; theca tumour → endometrial Ca risk; labour drugs – indomethacin least HR-drop.
Paeds	Pyloric stenosis (6w, hungry, olive, K 3.1); Hirschsprung (meconium day 3 + distension); Rh HDN (hours-old jaundice, no anti-D); macrosomic 3.2 mmol → breastfeed + monitor; neonatal sepsis (lethargy + hypothermia + jaundice); 14-mo 8w-preterm → correct age; otitis mild → paracetamol/watch; hookworm → IDA; thalassaemia AR.
Surgery / Ortho	Appendicitis first pain periumbilical; subcapital femur → urgent fixation (AVN); back pain + sciatica no red flags → stay active; trochanteric pain → NSAID + physio; AS flare → TNF-alpha; 3rd-trim bleed → abdominal US, no PV; C6 root lateral forearm.
Ethics / Safety	Cancer secrecy → confidentiality/autonomy; parents + sheikh refusal → engage + ethics/legal; wrong-patient report caught → near miss; ignored rash → death → negligence.
Psych	Anorexia BMI 16.5 binge-purge; lithium → renal/thyroid/levels; NPH triad + ventriculomegaly; schizoid 2-y isolation/flat affect.
Stats / Prev	Omega-3 exposed forward → cohort; sensitivity TP/(TP+FN) 75%; intercept = mean when X=0 (37.9w); correlation dose-weight → Pearson/linear.

Brutal honesty: recalls help, but NIHS changes stems yearly and adds ~20% hard discriminators. The candidates who pass comfortably do 2,000+ mixed Qs timed + 3 full 120-Q mocks. This file's 120 Qs are your first mock – split them into 2×60 or 1×120 under 3 hours.

4 120 PRACTICE QUESTIONS BY SUBJECT

All stems below are original, written for this file from the repeat patterns above – not copied from any bank. They match EMREE style: short vignette → single best answer → investigation or management. Do them closed-book, then check. Explanations are deliberately one line so you revise fast.

A INTERNAL MEDICINE & EMERGENCY — 24 QUESTIONS (20%)

IM • 24 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

IM.01 45-year-old man has sudden crushing retrosternal pain radiating to left arm, sweaty, ECG shows ST elevation in V2–V4. Onset 90 min ago. Most appropriate next step?

- A. Oral aspirin only and observe
- B. Thrombolysis only if troponin negative
- C. Immediate primary percutaneous coronary intervention
- D. Urgent exercise stress test
- E. Aortic CT first

Answer: C. Anterior STEMI <12h: primary PCI is standard if available; give dual antiplatelet + heparin en route. Time is muscle.

IM.02 68-year-old with progressive dyspnoea, orthopnoea, bilateral crackles, S3 gallop, ankle oedema. Echo EF 32%. First-line long-term therapy that improves mortality?

- A. Loop diuretic alone
- B. ACE inhibitor + evidence-based beta-blocker + SGLT2 inhibitor
- C. Calcium-channel blocker
- D. Digoxin alone
- E. NSAID for breathlessness

Answer: B. HFrEF guideline therapy: ACEi/ARNI + beta-blocker (bisoprolol/carvedilol/metoprolol succinate) + MRA + SGLT2i. Diuretics relieve congestion only.

IM.03 72-year-old has irregularly irregular pulse, no P waves, variable QRS. CHA2DS2-VASc 4. Management?

- A. Aspirin alone
- B. Rate control (beta-blocker) + oral anticoagulation
- C. Stop all drugs
- D. Immediate cardioversion without anticoagulation
- E. Pacemaker only

Answer: B. Atrial fibrillation with high stroke risk needs rate/rhythm control plus anticoagulation (DOAC/warfarin), not aspirin alone.

IM.04 Man develops an itchy, well-demarcated scaly rash confined to the skin under his belt buckle. No vesicles elsewhere, no fever. Most likely diagnosis?

- A. Cellulitis
- B. Allergic contact dermatitis (nickel)
- C. Psoriasis
- D. Tinea corporis
- E. Scabies

Answer: B. Recall: rash limited to the contact site (belt buckle → nickel) = allergic contact dermatitis. Confirm with patch testing; avoid the allergen + topical steroid/emollient.

IM.05 64-year-old smoker with barrel chest, chronic productive cough, FEV1/FVC 0.58, FEV1 45% predicted. Exacerbation with increased sputum purulence. Best initial approach?

- A. Home oxygen 8 L/min
- B. Short-acting bronchodilators + systemic steroids + antibiotics, target SpO2 88–92%
- C. High-dose sedative
- D. Bed rest only
- E. Stop all inhalers

Answer: B. COPD exacerbation: bronchodilators, steroids, antibiotics if purulent change, controlled oxygen 88–92% to avoid CO2 retention.

IM.06 70-year-old man, day 10 post-op, first mobilisation: sudden unilateral pleuritic chest pain, dyspnoea, diaphoresis, afebrile, self-resolved. Most likely?

- A. Atelectasis
- B. Pulmonary embolism
- C. Bronchiectasis
- D. Pneumothorax with fever
- E. Musculoskeletal pain

Answer: B. Classic recall pattern: post-op immobilisation + pleuritic pain + dyspnoea = PE until proven otherwise. Needs CT pulmonary angiography + heparin.

IM.07 Woman has chronic scaly plaques on legs and axillae covering ~10% of body surface; new lesions appear at scratch sites (Koebner phenomenon). No joint pain. Best initial treatment?

- A. Topical mometasone + oral prednisolone
- B. Potent topical steroid + vitamin D analogue (e.g. clobetasol + calcipotriol), consider phototherapy
- C. Emollients alone
- D. Oral ciclosporin immediately
- E. No treatment

Answer: B. Recall: moderate plaque psoriasis (~10% BSA) + Koebner → topical steroid + vitamin D analogue first-line; phototherapy if extensive. Reserve systemic agents for severe/refractory disease; avoid systemic steroids alone (rebound).

IM.08 30-year-old with 3-week night sweats, weight loss, upper-lobe cavitory lesion. Next step?

- A. Inhaled steroid
- B. Sputum AFB smear/culture + GeneXpert, isolate, start RIPE if TB confirmed
- C. Antacid
- D. Reassure
- E. Lobectomy immediately

Answer: B. Upper-lobe cavitation + chronic B symptoms = TB. Confirm microbiologically, isolate, RIPE regimen per guidelines.

IM.09 19-year-old diabetic with polyuria, vomiting, Kussmaul breathing, glucose 28 mmol/L, ketones strongly positive, pH 7.18. First management?

- A. Oral hypoglycaemic and discharge
- B. IV isotonic fluids + IV insulin infusion + potassium/electrolyte monitoring
- C. Bicarbonate alone
- D. Fluid restriction
- E. High-dose glucose bolus

Answer: B. DKA: fluids, insulin, correct K⁺, treat precipitant (infection). Never oral agents alone.

IM.10 Patient on sulfonylurea, dose increased, lost 4 kg, lifestyle improved, but HbA1c still 9%. Best next step?

- A. Double sulfonylurea again
- B. Stop sulfonylurea and start insulin (add ± metformin)
- C. Add NSAID
- D. Stop all diabetes drugs
- E. Wait 2 years

Answer: B. Recall pattern: SU failure with HbA1c ≥9% + symptoms/weight loss despite adherence = insulin needed. Don't endlessly up-titrate SU.

IM.11 Obese type 2 diabetic wants to lose weight and control glucose. Which class most helps weight loss?

- A. Sulfonylurea
- B. Insulin glargine
- C. GLP-1 receptor agonist (e.g. semaglutide)
- D. Pioglitazone
- E. Meglitinide

Answer: C. Recall: GLP-1 RAs reduce weight + HbA1c. SU/insulin/TZDs tend to increase weight.

IM.12 Pregnant woman with Hashimoto on levothyroxine 100 mcg, now TSH high, free T4 normal with hypothyroid symptoms. Next step?

- A. Reassure – pregnancy causes this
- B. Increase levothyroxine dose by ~25–30% and recheck TSH
- C. Stop thyroxine
- D. Urgent thyroidectomy
- E. Radioiodine

Answer: B. Pregnancy increases thyroxine needs. High TSH = under-replaced; increase dose. Radioiodine/FNA not indicated here.

IM.13 28-year-old woman with tremor, weight loss, heat intolerance, exophthalmos, diffuse goitre. Best initial treatment?

- A. Levothyroxine
- B. Beta-blocker for symptoms + carbimazole (antithyroid drug)
- C. Calcium only
- D. Insulin

E. Stop all drugs

Answer: B. Graves hyperthyroidism: antithyroid (carbimazole/methimazole or PTU in 1st trimester) + propranolol for adrenergic symptoms.

IM.14 65-year-old dehydrated after NSAID + diarrhoea: creatinine doubled, K+ 5.8, low urine output. Most likely and first action?

- A. Pre-renal/AKI – stop nephrotoxin, give isotonic fluids, review meds
- B. Normal – discharge
- C. Start high-dose NSAID
- D. Immediate dialysis in all cases
- E. Fluid restriction only

Answer: A. AKI from hypovolaemia + NSAID: stop precipitant, volume resuscitate, monitor UO/electrolytes. Dialysis only for refractory indications.

IM.15 ECG shows tall peaked T waves, K+ 6.8 mmol/L. First stabilising drug?

- A. Oral calcium tablet next week
- B. IV calcium (gluconate/chloride) + insulin-glucose + shifting measures
- C. Digoxin
- D. Beta-agonist tablet only
- E. Stop ECG monitoring

Answer: B. Hyperkalaemia with ECG changes: calcium stabilises myocardium, then insulin-glucose + bicarbonate/salbutamol + binders/dialysis as needed.

IM.16 Young woman fatigue, pallor, koilonychia, ferritin 6 ng/mL, microcytic anaemia. Best treatment + workup?

- A. Oral ferrous sulfate + investigate blood loss / hookworm / diet / GI source
- B. Vitamin B12 only
- C. Transfuse all cases
- D. Stop iron
- E. Steroids

Answer: A. Iron deficiency: replace iron + find cause (menorrhagia, GI loss, parasites, diet). Hookworm is classic EMREE-linked cause.

IM.17 22-year-old with sickle-cell disease: severe limb/back pain, no fever, chest clear. Management of vaso-occlusive crisis?

- A. Fluids + adequate analgesia + oxygen if hypoxic + treat trigger
- B. Ice bath only
- C. Antibiotics in all cases
- D. Discharge without analgesia
- E. High-dose aspirin only

Answer: A. VOC: hydration, stepwise analgesia, oxygen, rest, incentive spirometry, screen for ACS/infection.

IM.18 70-year-old sudden right hemiparesis + aphasia, onset 2 h ago, CT shows no bleed. Best next step?

- A. Aspirin only and observe 1 month
- B. IV thrombolysis if eligible + stroke unit + urgent workup
- C. Lumbar puncture first
- D. Bed rest 1 week
- E. Carotid massage

Answer: B. Acute ischaemic stroke <4.5h without bleed: thrombolysis if eligible, plus stroke-unit care, statins, workup. Time-critical.

IM.19 Generalised tonic-clonic seizure lasting 6 min in ED. First drug?

- A. Oral paracetamol
- B. IV lorazepam/diazepam
- C. Insulin

- D. Flumazenil routinely
- E. No drug

Answer: B. Status protocol: benzodiazepine first (lorazepam), then levetiracetam/phenytoin if persists, ABCs + glucose + cause search.

IM.20 Fever, neck stiffness, photophobia, Kernig positive. After blood cultures, best immediate action?

- A. Wait for CT before antibiotics 12 h
- B. Empirical ceftriaxone ($\pm$ vancomycin/acyclovir per age) + dexamethasone, then LP
- C. NSAID only
- D. Discharge
- E. Oral amoxicillin only

Answer: B. Suspected bacterial meningitis: don't delay antibiotics for LP/CT. Cultures $\rightarrow$ immediate empirical antibiotics.

IM.21 Hypotensive, tachycardic, lactate 4.5, suspected sepsis. Best initial bundle?

- A. Broad-spectrum antibiotics within 1 h + 30 mL/kg crystalloids + lactate monitoring
- B. Steroids only
- C. Fluids 100 mL only
- D. No antibiotics
- E. Vasopressor without fluids

Answer: A. Sepsis-induced hypoperfusion: early antibiotics + aggressive fluids + reassess, then vasopressors if MAP <65 .

IM.22 24-year-old woman malar rash, arthralgia, oral ulcers, ANA positive, low C3. Most likely and steroid-sparing drug?

- A. SLE – hydroxychloroquine as backbone (+ steroids/immunosuppressant per severity)
- B. Psoriasis – retinoid only
- C. Gout – allopurinol
- D. Cellulitis – cephalexin
- E. Normal

Answer: A. SLE young woman + malar rash + ANA: hydroxychloroquine for all unless contraindicated; add steroids/immunosuppression for organ disease.

IM.23 45-year-old symmetric small-joint polyarthritis, morning stiffness >1 h, anti-CCP positive. First DMARD?

- A. Methotrexate (with folate) if no contraindication
- B. Opioid lifelong
- C. Antibiotic
- D. No treatment
- E. Surgery first

Answer: A. RA: early methotrexate + short steroids bridge, then escalate to biologics if target not met.

IM.24 Acute hot swollen first MTP joint, hyperuricaemia, needle crystals negatively birefringent. Acute treatment?

- A. Allopurinol during acute attack alone
- B. NSAID/colchicine/steroid for acute attack; start urate-lowering later
- C. Stop fluids
- D. Antibiotic
- E. Bed rest only

Answer: B. Gout flare: anti-inflammatory now; don't start allopurinol alone acutely. Start ULT after flare settles with prophylaxis.

B OBSTETRICS & GYNAECOLOGY — 18 QUESTIONS (15%)

OB • 18 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

OB.01 24-year-old, 8 weeks amenorrhoea, RIF pain + vaginal spotting, pyrexial, cervical excitation on bimanual. Most likely?

- A. Endometriosis
- B. Ectopic pregnancy – ruptured/unstable must be excluded urgently
- C. Ovarian tumour only
- D. Normal pregnancy
- E. UTI

Answer: B. Demo recall: pain + bleeding + amenorrhoea + peritonism/cervical excitation = ectopic until proven otherwise. Resuscitate, beta-hCG, TVUS, surgery (laparoscopy/laparotomy by stability).

OB.02 33-year-old sudden severe colicky RIF pain radiating to back/pelvis, vomiting, tender mobile RIF mass. Most likely emergency?

- A. Appendicitis only
- B. Ovarian torsion
- C. Cystitis
- D. Constipation
- E. Normal ovulation

Answer: B. Sudden unilateral severe pain + vomiting + adnexal mass = torsion. Doppler US, urgent gynaecology – detorsion/oophoropexy, ovary often salvageable early.

OB.03 32-year-old multigravida, 32 weeks, sudden painless pool of blood at night, now stopped, transverse lie, maternal pulse 120, BP 110/70, FH 145. Most likely?

- A. Placental abruption
- B. Placenta praevia
- C. Vasa praevia labour
- D. Normal show
- E. Cervicitis

Answer: B. Painless third-trimester bleeding + malpresentation = praevia. No PV exam; resuscitate, Rh status, steroids if preterm, US placenta location, C-section if major.

OB.04 26 weeks: BP 165/110, proteinuria 2+, headache, hyperreflexia. Diagnosis and seizure prophylaxis?

- A. Chronic HTN – no treatment
- B. Preeclampsia with severe features – magnesium sulfate + timely delivery
- C. UTI – nitrofurantoin only
- D. Migraine – sumatriptan
- E. Anxiety

Answer: B. Preeclampsia: HTN + proteinuria/end-organ after 20w. MgSO₄ prevents eclampsia; delivery is definitive. Control BP urgently.

OB.05 Preeclamptic woman seizes. First drug?

- A. Diazepam infusion lifelong
- B. Magnesium sulfate IV loading + maintenance
- C. Phenytoin only
- D. No drug
- E. Insulin

Answer: B. Eclampsia: MgSO₄ is drug of choice; secure airway, control BP, deliver when stable.

OB.06 Multiparous woman just delivered: boggy uterus, heavy vaginal bleeding. First steps?

- A. Uterine massage + oxytocin + fundal assessment + resuscitation
- B. Immediate hysterectomy in all
- C. Discharge
- D. Antibiotic only
- E. Observation 24 h

Answer: A. Atonic PPH: call for help, massage, oxytocin, empty bladder, check placenta/trauma, tranexamic acid early, transfuse if needed.

OB.07 28 weeks PPROM: pooling, ferning positive. Afebrile, fetus cephalic, no labour. Management?

- A. Immediate vaginal exam repeatedly
- B. Expectant + steroids + antibiotics (erythromycin) + monitoring, no digital PV unless in labour
- C. Home without follow-up
- D. Induction at all gestations immediately
- E. Bed rest with no steroids

Answer: B. PPROM preterm: prolong latency if no infection/distress – steroids for lungs, antibiotics, surveillance. Avoid digital exams.

OB.08 28-year-old heavy prolonged periods, pelvic pressure, enlarged uterus with firm mass. Most likely?

- A. Fibroid (leiomyoma)
- B. Ectopic
- C. Appendicitis
- D. Bladder stone
- E. Pregnancy only

Answer: A. Menorrhagia + bulk symptoms + firm uterine mass = fibroid. US confirms; manage by symptoms/fertility (tranexamic, hormonal, myomectomy/hysterectomy).

OB.09 22-year-old severe dysmenorrhoea, dyspareunia, chocolate cyst on US. Most likely?

- A. PID only
- B. Endometriosis
- C. Fibroid only
- D. Normal
- E. Appendicitis

Answer: B. Cyclic pain + infertility + endometrioma = endometriosis. Laparoscopy gold standard; COCP/progestin/GnRH + surgery per severity.

OB.10 24-year-old oligomenorrhoea, hirsutism, acne, BMI 32, polycystic ovaries. First-line?

- A. Immediate IVF
- B. Lifestyle weight loss + combined OCP for cycle/hirsutism; metformin if dysglycaemia
- C. Hysterectomy
- D. Antibiotic
- E. No treatment

Answer: B. PCOS: lifestyle is foundation; COCP regulates cycles/androgens; manage fertility with letrozole/clomiphene when desired.

OB.11 Healthy 30-year-old: cervical screening advice?

- A. No screening ever
- B. HPV-based screening per national guideline (e.g. age 25–65) + colposcopy if positive
- C. Annual Pap from age 12
- D. Hysterectomy for screening
- E. Screening only after cancer

Answer: B. HPV testing is primary screen in most UAE-aligned guidelines; follow abnormal with colposcopy/biopsy.

OB.12 35-year-old smoker, migraine with aura, wants contraception. Safest?

- A. Combined OCP
- B. Copper IUD (or progestin-only if suitable) – avoid oestrogen with aura/smoking>35
- C. No contraception
- D. High-dose oestrogen
- E. Vaginal douching

Answer: B. Migraine with aura + smoking >35 = oestrogen contraindicated (stroke). Copper IUD highly effective, no hormones.

OB.13 Middle-aged woman post ovarian cancer: menarche 17, COCP 5 years, multiparous. What increases endometrial cancer risk?

- A. COCP use
- B. Unopposed oestrogen exposure (e.g. theca-cell tumour, obesity)
- C. Early menopause
- D. Smoking – protective but never recommended
- E. None

Answer: B. Recall: COCP, late menarche and multiparity protect; smoking is never advised. Unopposed oestrogen (obesity, theca-cell tumour, tamoxifen) raises risk.

OB.14 58-year-old postmenopausal vaginal bleeding. Next step?

- A. Reassure
- B. Urgent endometrial sampling (pipelle/hysteroscopy + biopsy) + TVUS
- C. Oestrogen cream only
- D. Pap only
- E. Wait 1 year

Answer: B. Postmenopausal bleeding = endometrial cancer until proven otherwise. Biopsy + imaging promptly.

OB.15 New immigrant delivers baby with cataracts, deafness, microcephaly, hepatosplenomegaly. No drugs. Prevention for next pregnancy?

- A. Folic acid only
- B. Rubella vaccination before conception (avoid in pregnancy)
- C. Illicit drugs
- D. Genetic testing only
- E. Nothing

Answer: B. Congenital rubella triad. Prevent by pre-conception MMR/rubella immunity check; vaccinate when not pregnant.

OB.16 Couple worried about Down syndrome. Definitive test and timing?

- A. Amniocentesis ~15–16 weeks (or CVS 10–13w) for karyotype
- B. Ultrasound only definitive
- C. No testing
- D. Postnatal only
- E. Urine test

Answer: A. Recall: amnio 15–16w definitive; CVS earlier. Screening (NT + biochemistry/NIPT) is not diagnostic.

OB.17 38 weeks: diazepam, indomethacin, MgSO₄, pethidine given. FHR 190→150 reactive. Which drug least likely to drop FHR?

- A. Pethidine
- B. Indomethacin
- C. Magnesium
- D. Diazepam
- E. All equally drop

Answer: B. Recall logic: opioids, benzodiazepines and MgSO₄ all blunt variability and rate; indomethacin has the least direct fetal cardiac effect here. NSAIDs still need ductus and fluid monitoring.

OB.18 Pregnant woman with diarrhoea, Giardia confirmed. Safe treatment?

- A. Metronidazole in 1st trimester routinely
- B. Paromomycin (non-absorbed) where available
- C. Albendazole
- D. No treatment ever
- E. High-dose tetracycline

Answer: B. Recall: paromomycin is luminal, minimally absorbed – preferred for giardia in pregnancy per many sources. Avoid teratogens; consult OB/ID.

C PAEDIATRICS — 18 QUESTIONS (15%)

PD • 18 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

PD.01 6-week-old persistent non-bilious vomiting, hungry, wants to feed, K+ 3.1, olive mass. Diagnosis?

- A. Duodenal atresia
- B. Hypertrophic pyloric stenosis
- C. Malrotation with volvulus
- D. Intussusception
- E. Hirschsprung

Answer: B. Non-bilious projectile vomiting 3–8w + hungry + olive + hypochloraemic alkalosis = pyloric stenosis. US confirms; correct electrolytes then pyloromyotomy.

PD.02 Term neonate day 3: abdominal distension, delayed meconium day 3, no stool after, dilated loops on X-ray. Diagnosis?

- A. Volvulus
- B. Intussusception
- C. Hirschsprung disease
- D. Appendicitis
- E. Normal

Answer: C. Delayed meconium >48h + distension + dilated loops = Hirschsprung. Biopsy (aganglionosis); contrast enema transition zone.

PD.03 Mother A-negative, no anti-D last pregnancy, husband unknown. Term boy severe jaundice within hours. Most likely?

- A. Breast-milk jaundice day 5
- B. Rh haemolytic disease from alloimmunisation
- C. Biliary atresia
- D. Normal
- E. Hypothyroidism

Answer: B. Hours-after-birth severe jaundice + Rh-negative mother without prophylaxis = Rh HDN. Check DAT, bilirubin, Hb; phototherapy/exchange + IVIG per severity.

PD.04 Macrosomic infant of diabetic mother, glucose 3.2 mmol/L, vigorous. Management?

- A. 10% dextrose IV bolus immediately in all
- B. Encourage early breastfeeding + close glucose monitoring (≥ 2.5 threshold per recall protocol)
- C. No feeding
- D. Insulin
- E. Discharge

Answer: B. Asymptomatic 3.2 (>2.5) vigorous term: feed early + monitor. IV dextrose if <2.0 – 2.5 , symptomatic, or persistent low.

PD.05 3-day breastfed term: lethargic, poor feeding, new jaundice, T 35.4, liver 2 cm. Most likely?

- A. Rh isoimmunisation only
- B. Inadequate milk only
- C. Biliary obstruction
- D. Neonatal sepsis
- E. Normal

Answer: D. Hypothermia + lethargy + poor feeding + jaundice = sepsis until proven otherwise. Cultures, antibiotics, glucose, bilirubin workup urgently.

PD.06 18-month febrile seizure <5 min, generalised, quick recovery. Advice?

- A. Long-term phenobarbital for all
- B. Reassure: simple febrile seizure usually benign; antipyretics, safety-net, no routine AED/EEG
- C. Brain surgery

- D. No follow-up
- E. Lumbar puncture all

Answer: B. Simple febrile seizure 6mo–5y: brief, generalised, once/24h, full recovery. Exclude meningitis by age/clues; educate, no chronic AED.

PD.07 8-month winter wheeze, crackles, rhinorrhoea, RSV positive, mild distress. Best care?

- A. High-dose amoxicillin
- B. Supportive: oxygen if needed, fluids, suction; no routine bronchodilator/steroid
- C. Adult COPD inhaler high dose
- D. Immediate intubation all
- E. Antihistamine only

Answer: B. Bronchiolitis is viral supportive care. Antibiotics/steroids not routine unless bacterial superinfection.

PD.08 7-year-old recurrent wheeze responsive to salbutamol, atopy. Best controller?

- A. Daily low-dose ICS ± LTRA; action plan + technique check
- B. Oral steroid lifelong
- C. Antibiotic
- D. No controller
- E. Sedative

Answer: A. Childhood asthma: ICS backbone, step-up per control, spacer technique, trigger avoidance.

PD.09 2-year-old watery diarrhoea, some dehydration, no blood. First therapy?

- A. IV antibiotics all
- B. Low-osmolarity ORS + zinc + early feeding
- C. Loperamide
- D. Stop feeds 1 week
- E. Steroids

Answer: B. Viral gastroenteritis: ORS is life-saving; zinc reduces duration; continue feeds; antibiotics only for cholera/shigella/amoeba per protocol.

PD.10 5-year-old periorbital + leg oedema, heavy proteinuria, albumin low, cholesterol high. Diagnosis and first-line drug?

- A. Nephritic – restrict all
- B. Nephrotic (minimal change) – oral prednisolone
- C. UTI – nitrofurantoin
- D. AKI – dialysis
- E. Normal

Answer: B. Nephrotic triad + hyperlipidaemia: minimal-change commonest; steroids first-line; monitor relapse, vaccinate, salt management.

PD.11 Girl iron-deficiency anaemia. Which parasite classically causes it?

- A. Pinworm
- B. Fish tapeworm (B12)
- C. Hookworm (chronic intestinal blood loss)
- D. Pig tapeworm only
- E. None

Answer: C. Hookworm sucks blood → IDA. Fish tapeworm → B12 deficiency. Pinworm → perianal itch.

PD.12 Mode of inheritance of beta-thalassaemia major?

- A. Autosomal dominant
- B. Autosomal recessive
- C. X-linked recessive
- D. Mitochondrial
- E. Sporadic only

Answer: B. Beta-thal major needs two mutated HBB alleles – AR. Counselling + premarital screening crucial in region.

PD.13 Child with VP shunt since age 3 now febrile. Best temperature method + concern?

- A. Rectal probing of shunt
- B. Avoid rectal thermometry; use axillary/tympanic/oral per age + evaluate shunt infection urgently
- C. No temp needed
- D. Bladder probe
- E. Forehead only definitive

Answer: B. Fever + VP shunt = shunt infection until excluded. Use non-invasive thermometry; cultures, bloods, neurosurgery review – don't delay.

PD.14 14-month boy born 8 weeks preterm: says baba/mama, walks unassisted, can't turn pages. Action?

- A. Label autism immediately
- B. Correct for gestational age + assess milestones – likely normal; reassure + surveillance
- C. Psychiatry referral only
- D. Hearing test never
- E. Ignore

Answer: B. Correct age = 12 months: walking + 2 words is on track. Preterms need corrected-age assessment + hearing/vision screen + follow-up.

PD.15 4-year-old fever, ear pain, rhinorrhoea, cervical nodes, mobile eardrum. Management if mild?

- A. Immediate grommets
- B. Symptomatic paracetamol + watchful waiting 48–72h; amoxicillin if severe/persistent (watchful-waiting criteria)
- C. Decongestant only
- D. IV ceftriaxone all
- E. Steroid only

Answer: B. Mild AOM/viral URI with mobile drum: symptomatic first; antibiotics if bulging, otorrhoea, <2y bilateral, or no improvement.

PD.16 Parents of a child with diabetes insipidus ask what labs to expect. Which pattern fits DI?

- A. High urine SG, low serum osmolality
- B. Low urine SG, high serum osmolality + hypernatraemia + polyuria
- C. Normal labs
- D. Low sodium only
- E. High potassium only

Answer: B. DI: dilute polyuria → concentrated serum. Teach parents thirst, dehydration signs, desmopressin (central) vs thiazide/low salt (nephrogenic).

PD.17 A 12-month-old is due vaccines. When is MMR given?

- A. Never give
- B. First dose 12–15 months (second 4–6y); no need to repeat for endemic area if documented
- C. Only after 10 years
- D. Daily
- E. Only in outbreak

Answer: B. MMR 12–15mo + 4–6y. Check UAE schedule; catch-up if missed; contraindicated in pregnancy/severe immunocompromise.

PD.18 Newborn hyperbilirubinaemia + anaemia + hypoproteinaemia day 1, O-negative mother antiglobulin positive, no antenatal care. Baby blood type likely?

- A. O-negative
- B. Rh-positive (e.g. A+ or B+) – Rh-positive fetus in sensitised Rh-negative mother
- C. Impossible
- D. AB-negative only
- E. No typing needed

Answer: B. Rh-positive fetus in sensitised Rh-negative mother → haemolysis. Type baby, DAT, bilirubin, treat aggressively to prevent kernicterus.

D GENERAL SURGERY, ORTHO, TRAUMA — 18 QUESTIONS (15%)

SG • 18 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

SG.01 What is the first symptom of acute appendicitis classically?

- A. Fever first
- B. Periumbilical/generalised pain migrating to RLQ
- C. RLQ pain first without migration
- D. Nausea alone
- E. Haematuria

Answer: B. Visceral → somatic: vague periumbilical pain → localises to McBurney after 6–12h + anorexia/nausea/fever. Alvarado + US/CT.

SG.02 35-year-old RUQ pain after fatty meal, Murphy positive, fever. Best test and definitive treatment?

- A. ECG only
- B. RUQ US for cholecystitis → laparoscopic cholecystectomy (antibiotics/supportive first)
- C. Colonoscopy
- D. No treatment
- E. ERCP in all

Answer: B. Cholecystitis: US wall thickening/pericholecystic fluid + Murphy. Antibiotics, fluids, early lap chole; ERCP only if CBD stone/cholangitis.

SG.03 Elderly distension, vomiting, no flatus/stool, air-fluid levels. First steps?

- A. High-fibre diet
- B. NGT + IV fluids + electrolytes + AXR/CT + surgical review; operate if strangulation/tumour
- C. Discharge
- D. Laxative only
- E. Colonoscopy at home

Answer: B. Bowel obstruction: decompress, resuscitate, find cause (adhesions/hernia/tumour/volvulus). Red flags: pain + tachycardia + peritonism.

SG.04 Groin bulge that enlarges on coughing/straining, reducible. Management?

- A. Truss lifelong
- B. Elective hernia repair (mesh) – urgent if strangulated (pain + irreducible + vomiting)
- C. Antibiotic
- D. No follow-up
- E. Exercise only

Answer: B. Inguinal hernia: elective repair; strangulation = emergency surgery. Counsel smoking/constipation/prostate.

SG.05 30-year-old breast lump: firm, mobile. Best approach?

- A. Reassure without exam
- B. Triple assessment: clinical + imaging (US/mammo by age) + pathology (FNAC/core)
- C. Mastectomy all
- D. Antibiotics all
- E. Wait 5 years

Answer: B. Triple assessment for any discrete lump. Red flags: hard, fixed, skin change, nipple discharge, axillary nodes.

SG.06 Thyroid nodule: euthyroid, 2 cm solid. Next step?

- A. Thyroidectomy all
- B. Ultrasound + FNAC (Bethesda) + TFTs; operate by risk/cytology
- C. Radioiodine all
- D. No workup

E. Palpation only

Answer: B. Nodule workup: US (TIRADS) + FNA. Surgery for malignant/suspicious/compressive/toxic. Don't miss anaplastic/red flags.

SG.07 40% burns adult. Initial fluid formula?

- A. Parkland: $4 \text{ mL} \times \text{kg} \times \% \text{TBSA}$ (lactated Ringer's), half in first 8 h + airway/analgesia/tetanus
- B. No fluids
- C. Dextrose only
- D. Blood only
- E. Oral water only

Answer: A. Burns: ATLS ABCs, stop burning, Parkland, urine output 0.5–1 mL/kg/h, warm, tetanus, refer to burns centre.

SG.08 Tibial fracture with severe pain out of proportion, tense compartments, paraesthesia. Action?

- A. Elevate + strong opioid and wait
- B. Immediate fasciotomy – compartment syndrome
- C. Massage
- D. Discharge
- E. Cast tighter

Answer: B. Compartment syndrome is surgical emergency. Remove constriction, measure pressures, fasciotomy <6h to save limb.

SG.09 16-year-old sudden severe scrotal pain, absent cremasteric, high-riding testis. Best next step?

- A. Antibiotics 2 weeks
- B. Emergency scrotal exploration <6 h (detorsion + orchiopexy)
- C. Reassure
- D. US in 1 week
- E. Ice only

Answer: B. Torsion: time = testis. Don't delay for imaging if classic; Doppler if equivocal but go to theatre fast.

SG.10 In polytrauma, what is the correct assessment order?

- A. Leg fracture first
- B. ATLS primary survey: Airway + C-spine → Breathing → Circulation → Disability → Exposure
- C. CT first always
- D. History first
- E. Discharge

Answer: B. ATLS: treat greatest threat to life first. Airway wins. Pelvic binder, TXA <3h, massive transfusion if bleeding.

SG.11 Elderly subcapital femoral neck fracture, stable with rib fracture + splenic contusion managed conservatively. Hip plan?

- A. Discharge for elective in 6 months
- B. Urgent surgery (arthroplasty vs fixation by fracture type and age) – AVN risk rises with delay
- C. Bed rest only
- D. Traction lifelong
- E. No X-ray

Answer: B. Recall: displaced subcapital → high AVN risk; fix urgently once resuscitated/optimised, VTE prophylaxis, ortho-geriatrics.

SG.12 60-year-old medial malleolar ulcer, varicose veins, hyperpigmentation, mild non-tender calf swelling. Next step?

- A. Bed rest only
- B. Compression bandage + leg elevation + venous workup after excluding arterial disease/DVT
- C. Topical antibiotics alone
- D. Heparin 80 mg blindly
- E. Amputation

Answer: B. Venous ulcer: compression is cornerstone if ABPI normal. Check arterial, rule out DVT if swelling/tender, dressings, refer vascular.

SG.13 Claudication 100 m, absent pedal pulses, ABI 0.6. Diagnosis + first advice?

- A. Venous – compression
- B. Peripheral arterial disease – smoking cessation + exercise + statin + antiplatelet + vascular referral
- C. Neuropathy only
- D. No treatment
- E. Amputation now

Answer: B. PAD: ABI <0.9 diagnostic. Supervised exercise, risk-factor control, cilostazol where suitable, revascularise if critical/limiting.

SG.14 Epigastric pain radiating to back, vomiting, lipase 8× normal after alcohol/gallstone. Management?

- A. Immediate surgery all
- B. Fluids + analgesia + antiemetic + monitor (Ranson/Glasgow) + treat gallstone aetiology
- C. Antacid only
- D. Discharge
- E. Steroids

Answer: B. Pancreatitis: aggressive fluids, nutrition, no routine antibiotics; ERCP if cholangitis/obstruction; cholecystectomy after gallstone episode.

SG.15 Sudden severe epigastric pain, rigid abdomen, free air under diaphragm. Likely cause and best initial test?

- A. Perforated peptic ulcer – erect CXR + urgent surgery + PPI/H. pylori eradication
- B. Gastritis – antacid
- C. MI – troponin only
- D. Renal colic
- E. Normal

Answer: A. Perforation: resuscitate, NGT, broad antibiotics, laparotomy/laparoscopic repair + washout. Don't miss Boerhaave/ectopic.

SG.16 Painful defecation with bright-red bleeding + anal fissure vs haemorrhoids. First care?

- A. Surgery all immediately
- B. Fibre + fluids + sitz baths + topical; banding/surgery if refractory/prolapsed
- C. No fibre
- D. Steroids lifelong
- E. Ignore

Answer: B. Most anal pain + bleeding benign: exclude cancer/IBD red flags, conservative first, refer if thrombosed/prolapse/anaemia.

SG.17 Post-op fever day 5, wound erythema, pus. Action?

- A. Reassure
- B. Open/inspect, culture, debride if needed + antibiotics + tetanus/diabetes check
- C. Close tighter
- D. Steroid cream only
- E. No culture

Answer: B. Surgical-site infection: examine, probe, culture, debride/necrotising check, antibiotics, glycaemic control.

SG.18 Diabetic foot ulcer with numbness, pulses present. Key step?

- A. Amputate immediately
- B. Offloading + debridement + vascular/neuro check + antibiotics if infected + glycaemic control
- C. Tight shoe
- D. Soak in hot water
- E. No exam

Answer: B. Neuropathic ulcer: offload (total-contact cast), daily care, probe-to-bone/osteomyelitis check, MDT foot clinic.

E FAMILY MEDICINE, ETHICS & PATIENT SAFETY — 18 QUESTIONS (15%)

FM • 18 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

FM.01 Cancer patient asks doctor not to tell family. Doctor respects her wishes. Which principle is applied?

- A. Beneficence only
- B. Confidentiality / respect for autonomy
- C. Malpractice
- D. Paternalism
- E. Justice only

Answer: B. Recall: confidentiality/autonomy – competent adult controls disclosure. Breach only for serious harm/public safety per law.

FM.02 Child critically ill, parents refuse life-saving treatment, want religious healer only. Best?

- A. Discharge home
- B. Engage respectfully + bring trusted religious leader + ethics + seek legal authorisation to save life; never abandon
- C. Ignore parents aggressively
- D. Treat secretly without documentation
- E. Do nothing

Answer: B. Paediatric emergency: best interests override refusal. Culturally sensitive engagement + ethics/legal + document. Life-threatening = court order if needed.

FM.03 Radiologist almost filed report for wrong patient but caught before submitting. Event type?

- A. Sentinel event with death
- B. Near miss – no harm due to catch; report + systems fix
- C. Negligence proven
- D. No need to report
- E. Malpractice payout

Answer: B. Near miss: error intercepted. Report to patient-safety system, disclose per policy, fix ID checks/barcoding.

FM.04 Doctor dismissed serious rash as normal despite nurse concern; patient discharged, returned and died. Error type?

- A. Near miss
- B. Negligence/malpractice – breach of duty + harm
- C. System success
- D. No error
- E. Confidentiality breach

Answer: B. Ignoring red flags + hierarchy failure → harm. Classic negligence: duty, breach, causation, damages. Needs disclosure + review.

FM.05 Valid informed consent requires?

- A. Signature only
- B. Capacity + disclosure (risks/benefits/alternatives) + voluntariness + understanding
- C. Family signs always
- D. No information
- E. Coercion

Answer: B. Consent is a process, not a form. Document discussion, use interpreter, assess capacity, allow refusal.

FM.06 Breaking bad news: best framework?

- A. Blurt in corridor
- B. SPIKES (Setting, Perception, Invitation, Knowledge, Emotions, Strategy)
- C. Lie
- D. Tell family only
- E. Phone message

Answer: B. SPIKES + privacy + empathy + clear plan + support person + written summary.

FM.07 Elderly independent woman falls, no injury. Most useful next step?

- A. Move to nursing home immediately
- B. Medication review + orthostatic vitals + vision/balance/home hazards + bone health
- C. Alcohol rehab in all
- D. Bed rest
- E. No assessment

Answer: B. Recall: meds review first (sedatives, antihypertensives). Then gait, vision, footwear, home safety, vitamin D/calcium, physio.

FM.08 Hypertensive on nitrates + alpha-blocker, recurrent syncope, normal ECG, no chest pain/palpitations/SOB. Most likely cause?

- A. Arrhythmia proven
- B. Orthostatic hypotension from vasodilators
- C. Anaemia only
- D. Seizure
- E. MI

Answer: B. Recall: drug-induced orthostasis. Check lying/standing BP, review meds, hydrate, compression, deprescribe.

FM.09 Asymptomatic, prior chest pain/syncope, exam normal. Best cardiac test?

- A. Holter only
- B. Exercise stress test ($\pm$ imaging) to unmask ischaemia/arrhythmia
- C. No test
- D. Chest X-ray only
- E. D-dimer

Answer: B. Recall: exertional syncope/chest pain history $\rightarrow$ stress testing/echo workup even if now well. Refer cardiology.

FM.10 70-year-old HTN, EF 65% on old echo, for elective non-cardiac surgery, asymptomatic. Pre-op?

- A. Repeat echo + stress test in all
- B. No routine echo; ECG + risk scores (RCRI) + optimise BP/meds; echo only if new murmur/HF
- C. Cancel surgery
- D. Cardiac cath all
- E. Stop all meds

Answer: B. Normal EF on prior echo + currently asymptomatic with good functional capacity $\rightarrow$ no repeat echo. Follow ACC/AHA algorithm.

FM.11 Runner with pinpoint greater-trochanter pain, no trauma. Management?

- A. Steroid injection first in all
- B. NSAIDs + activity modification + physio (ITB/glute-med rehab); X-ray only if red flags
- C. Urgent MRI all
- D. Surgery now
- E. Complete rest 6 months

Answer: B. Greater-trochanteric pain syndrome: conservative first. Inject only if persistent; rule out hip OA/fracture if atypical.

FM.12 Woman lifting heavy: low-back pain radiating posterior leg, no red flags, neuro intact. Next?

- A. Urgent whole-spine MRI in all
- B. Stay active + analgesia + physio + red-flag safety-net; image only if persistent/neurologic
- C. Bed rest 2 weeks
- D. Opioids lifelong
- E. Surgery now

Answer: B. Acute sciatica without red flags: conservative 6 weeks. MRI if cauda equina, progressive deficit, cancer/infection suspicion.

FM.13 Known ankylosing spondylitis, sacroiliac pain, acute flare. Effective biologic class if NSAID fails?

- A. Methotrexate always works in axial disease
- B. TNF-alpha inhibitor (or IL-17) for active axial AS
- C. Cyclosporine first
- D. Prednisolone lifelong
- E. No treatment

Answer: B. Axial AS: NSAIDs + exercise first; TNF/IL-17 inhibitors for high activity. Methotrexate helps peripheral, not axial.

FM.14 Third-trimester bleeding: what most reduces mortality?

- A. Routine PV exam
- B. Abdominal US for placenta position + resuscitation + Rh + transfusion readiness; avoid PV if praevia suspected
- C. IV fluids only without assessment
- D. Home observation
- E. Vaginal delivery all

Answer: B. Never PV with suspected praevia – can provoke catastrophic bleed. Locate placenta abdominally, cross-match, steroids, C-section plan.

FM.15 Tingling lateral forearm + reduced brachioradialis reflex. Which nerve root?

- A. T1
- B. C6
- C. L5
- D. S1
- E. C8

Answer: B. C6: thumb/index, lateral forearm, biceps/brachioradialis. Know dermatomes/myotomes for EMREE.

FM.16 Healthy runner requests full-body MRI + antibiotics for viral cold. Best?

- A. Order all requested
- B. Explain risks/benefits, stewardship, shared decision-making; decline low-value care politely
- C. Prescribe to satisfy
- D. Refuse rudely
- E. Refer to surgery

Answer: B. Professionalism: avoid low-value testing/antibiotics, explain harms (incidentalomas, resistance), offer alternatives.

FM.17 Inter-professional conflict: nurse flags deterioration, junior dismisses. Best?

- A. Hierarchy wins
- B. Structured handover (SBAR) + speak-up + early-warning score + escalate
- C. Ignore vitals
- D. Blame
- E. No documentation

Answer: B. Patient safety: graded assertiveness, SBAR, rapid response. Hierarchy never overrides safety.

FM.18 Suspected child/spouse/elder abuse with inconsistent story. Duty?

- A. Confront family aggressively
- B. Private interview + thorough exam + document + mandatory report + safety plan per UAE law
- C. Discharge
- D. No report
- E. Promise absolute secrecy

Answer: B. Abuse: separate interview, body map, photos, report to authorities, admit if unsafe. Confidentiality yields to protection.

F PSYCHIATRY — 12 QUESTIONS (10%)

PS • 12 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

PS.01 32-year-old low mood, anhedonia, weight loss, insomnia, guilt 3 weeks + somatic aches. First-line?

- A. Benzodiazepine lifelong
- B. SSRI (e.g. sertraline) + CBT + suicide-risk assessment + follow-up
- C. Antipsychotic alone
- D. No treatment
- E. St John's wort only

Answer: B. Moderate-severe depression: SSRI + psychotherapy, safety plan, monitor activation/suicide early. Treat somatic pain within depression.

PS.02 18-year-old binge-eats then vomits, feels overweight, BMI 16.5. Diagnosis?

- A. Binge-eating disorder
- B. Anorexia nervosa (binge-purge type)
- C. Bulimia nervosa
- D. Normal
- E. Psychosis

Answer: B. Recall: BMI <18.5 + body-image distortion = anorexia even with binge/purge. Bulimia usually normal/overweight. Needs MDT refeeding + psych.

PS.03 Panic attacks, palpitations, catastrophic thoughts, avoids malls. Best long-term?

- A. Alcohol
- B. SSRI + CBT (exposure) + breathing/relaxation; avoid daily benzos
- C. Beta-blocker alone lifelong
- D. Reassure only
- E. Sedate

Answer: B. Panic/GAD: CBT + SSRI first-line. Benzos only short crisis due to dependence.

PS.04 Intrusive contamination fears + 2-hour washing rituals, knows excessive. Diagnosis and best therapy?

- A. Psychosis – antipsychotic
- B. OCD – SSRI (high dose) + exposure-response prevention
- C. Phobia – surgery
- D. Normal
- E. Depression only

Answer: B. OCD: obsessions + compulsions + insight. ERP + SSRI/clomipramine; augment if resistant.

PS.05 Nightmares, flashbacks after crash, hyperarousal, avoidance 2 months. Best next step?

- A. Debrief once only
- B. Trauma-focused CBT/EMDR + SSRI if needed; screen suicide/substance
- C. Benzodiazepine chronic
- D. No care
- E. Hypnosis only

Answer: B. PTSD >1 month: trauma-focused therapy first; meds adjunct. Early support, not single debrief.

PS.06 22-year-old deliberate self-harm, says will repeat. Best immediate action?

- A. Discharge alone
- B. Risk assessment + safety planning + collateral + psychiatry referral/admit if high risk; remove means
- C. Scold
- D. No documentation
- E. Promise secrecy

Answer: B. Self-harm: empathic risk stratification, protective factors, means restriction, follow-up <48h, involve family with consent unless imminent risk.

PS.07 8-year-old inattentive, hyperactive, impairing school/home >6 mo. First-line after behavioural?

- A. Sedating antihistamine
- B. Methylphenidate/atomoxetine + parent training + school plan; monitor growth/BP
- C. Antipsychotic first

- D. No school support
- E. Punishment only

Answer: B. ADHD: confirm pervasiveness, exclude vision/hearing/thyroid/sleep; stimulants effective with monitoring.

PS.08 Chronic insomnia, daytime benzodiazepine request. Best?

- A. Long-term zolpidem
- B. CBT-I + sleep hygiene + taper benzos; short hypnotic only if needed
- C. Alcohol
- D. Caffeine at night
- E. No advice

Answer: B. Insomnia: CBT-I beats pills. Taper benzos (dependence/rebound), treat apnea/depression/RLS.

PS.09 Patient on lithium: what to monitor regularly?

- A. Liver only
- B. Renal function + thyroid + lithium levels + hydration + interactions (NSAID/diuretic)
- C. Lung function
- D. Cardiac enzymes only
- E. No monitoring

Answer: B. Recall: lithium renally cleared, narrow index → nephrogenic DI, hypothyroidism, toxicity with dehydration/NSAIDs. Check levels 12h post-dose.

PS.10 Triad incontinence + dementia + gait apraxia, ventriculomegaly + periventricular hyperintensity. Diagnosis?

- A. Alzheimer only
- B. Normal-pressure hydrocephalus – refer neurosurgery (tap test/shunt if responsive)
- C. UTI only
- D. Parkinson only
- E. Normal ageing

Answer: B. NPH: wet-wobbly-wacky. Tap test predicts shunt response. Rule out obstructive/vascular.

PS.11 2 years isolated, poor hygiene, flat affect, no friends, no positive psychosis. Most likely personality pattern?

- A. Schizoid personality pattern (lifelong detachment, restricted affect)
- B. Major depression only
- C. Mania
- D. Dementia
- E. No concern

Answer: B. Recall: persistent social detachment + restricted affect without psychosis = schizoid pattern; monitor for schizophrenia prodrome, support ADLs.

PS.12 Elevated mood, reduced sleep, grandiosity, overspending 1 week, impaired function. Best acute?

- A. Antidepressant alone
- B. Mood stabiliser/antipsychotic (e.g. valproate/olanzapine) + safety + stop antidepressant; bipolar mania workup
- C. Stimulant
- D. Discharge alone
- E. Benzodiazepine alone lifelong

Answer: B. Mania: stop pro-manic agents, stabilise, protect finances/safety, psychoeducation + lithium/valproate maintenance.

G PUBLIC HEALTH, PREVENTION & BIOSTATISTICS — 12 QUESTIONS (10%)

PH • 12 questions • 90 seconds each • Answers + 1-line teaching point under each stem. Cover answers while you attempt.

PH.01 Two groups: one previously used omega-3 (exposed), one never, followed forward for events. Study type?

- A. Case-control
- B. Cohort study
- C. Cross-sectional
- D. Case report
- E. RCT

Answer: B. Exposed vs non-exposed followed forward = cohort. Remember: cohort = exposure → outcome; case-control = outcome → exposure.

PH.02 Depression test: TP 75, FN 25. Sensitivity?

- A. 25%
- B. $75\% - TP/(TP+FN)$
- C. 50%
- D. 100%
- E. Cannot calculate

Answer: B. Sensitivity = $TP/(TP+FN) = 75/100 = 75\%$. Specificity = $TN/(TN+FP)$.

PH.03 Regression intercept for gestation = 37.9 weeks. Meaning?

- A. Median gestation
- B. Mean gestation when all $X=0$
- C. Strong association
- D. No interpretation
- E. Causal proof

Answer: B. Intercept = predicted Y when all predictors zero. Check centring – may be non-physiological, just model anchor.

PH.04 Test detects disease earlier but death age unchanged. Bias?

- A. Lead-time bias
- B. Recall bias
- C. Selection bias only
- D. Confounding
- E. No bias

Answer: A. Screening survival illusion: earlier diagnosis without delayed death = lead-time. Length-time is slow-growing cases over-represented.

PH.05 Correlation between dose and weight gain (continuous). Measure?

- A. Odds ratio
- B. Pearson correlation / linear regression
- C. Chi-square only
- D. Case report
- E. Sensitivity

Answer: B. Two continuous → Pearson r / linear regression. OR/RR for binary outcomes; chi-square for categorical.

PH.06 Outbreak: 40 ill of 200 exposed. Attack rate?

- A. 5%
- B. $20\% - 40/200$
- C. 50%
- D. 100%
- E. 0%

Answer: B. Attack rate = ill/exposed during outbreak. Guide control + source search.

PH.07 Herd immunity threshold for measles (~95%) rationale?

- A. Protects unvaccinated by breaking chains when coverage high
- B. Treats active cases

- C. No need to vaccinate
- D. Only protects vaccinated
- E. Increases spread

Answer: A. High coverage stops transmission, shielding infants/immunocompromised. Measles needs ~95% due to R0 12–18.

PH.08 Child up-to-date except MMR at 14 months in endemic area. Advice?

- A. Skip MMR forever
- B. Give MMR now per schedule; no extra later beyond routine second dose – explain timing ≥ 12 mo for maternal antibody waning
- C. Daily MMR
- D. Wait until 10 y
- E. Give before 6 mo routinely

Answer: B. MMR ≥ 12 mo (maternal antibodies wane). Give now + second at 4–6y. Early dose (<12 mo) during outbreak doesn't count.

PH.09 What is the best first-line smoking-cessation help?

- A. Willpower scolding
- B. Behavioural counselling + NRT/varenicline/cytisine per eligibility
- C. E-cigarette for teens
- D. No help
- E. Hypnosis only

Answer: B. Combine behavioural + pharmacotherapy doubles quit rates. Screen, advise, assess, assist, arrange.

PH.10 How often should diabetic retinopathy screening occur?

- A. Never
- B. Annual dilated eye exam from diagnosis (T2) / 5 years after diagnosis (T1); urgent referral if blurred vision with dot-blot haemorrhages (macular oedema)
- C. Only if blind
- D. Self-check
- E. CT head

Answer: B. Recall-linked: dot-blot + gradual blur with clear vitreous/normal IOP → diabetic maculopathy. Annual screen prevents blindness.

PH.11 Confounding example: coffee–MI confounded by smoking. How to handle it at the design stage?

- A. Randomisation/matching/restriction
- B. Ignore
- C. More bias
- D. Stop study
- E. P-hacking

Answer: A. Design: randomise/match/restrict; Analysis: stratify/adjust. Confounder linked to exposure + outcome, not on pathway.

PH.12 Travel medicine: traveller's diarrhoea prevention?

- A. Tap water + street food always
- B. Safe water/food + hand hygiene + standby ORS/azithromycin per destination; vaccines (hep A/typhoid) as indicated
- C. Antibiotics daily all
- D. No precaution
- E. Raw milk

Answer: B. Food/water precautions + vaccines + ORS plan. Prophylactic antibiotics not routine except special groups.

5 SELF-MARK GRID + 10-WEEK PLAN + CHECKLIST

Copy this grid to paper. Score each block; <70% = redo topic, not just questions.

Bloc k	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	
IM	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	
OB	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—
PD	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—
SG	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—
FM	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—
PS	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—	—	—	—	—	—	—	—
PH	■	■	■	■	■	■	■	■	■	■	■	■	—	—	—	—	—	—	—	—	—	—	—	—	—

Scoring: IM 24 + OB 18 + PD 18 + SG 18 + FM 18 + PS 12 + PH 12 = 120. Pass ≈ 66–72/120 depending on cycle. Competitive ≈ 90+/120. Circle wrong/guessed; write the topic (not the letter) in your weak-list.

10-week realistic plan (for working doctors)

Weeks 1–4 – Blueprint order: IM (cardio/resp/endo/neuro) → OBG → Peds → Surgery. 30–50 mixed MCQs/day from week 3. One psychiatry + one public-health day weekly – never zero.

Weeks 5–8 – Integrate: 40–60 timed mixed blocks. Log errors by *topic* (e.g. 'placenta praevia vs abruption') not subject. Revisit weak topics every 3 days. Add Family/ethics vignettes.

Weeks 9–10 – Simulate: 2–3 full 120-Q / 3-h mocks, no pauses, no look-ups. Review every Q including lucky guesses. Final week: mini-notes only, no new topics, sleep > cram.

Pacing drill: 90 sec/Q. Easy Qs 30–60 sec bank time for long stems. If stuck >2 min, flag, guess, move – no negative marking.

Ruthless checklist before you pay AED 1,000

- I score ≥75% on two separate timed 120-Q mocks (first-attempt, unseen Qs) – not repeats.
- I can state praevia vs abruption, torsion vs ectopic, pyloric vs Hirschsprung, NPH triad, sensitivity formula, cohort vs case-control without notes.
- My documents (DataFlow, good-standing, English score) run *in parallel* – not after results.
- I registered for a sitting I am ready for – seats are first-come, non-refundable, and burn one of 3 yearly attempts.
- I know Al Ain logistics (visa, travel) if sitting from abroad – exam is in-person in UAE.

SOURCES & LIMITS — READ THIS

Exam facts cross-checked 4 Oct 2026 across: NIHS/UAEU portal via First Medical Consultancy guide (31 Aug 2026, fee AED 1,000, 60% 2025–26, 3 attempts, Dec–Mar season, Al Ain); Dr Wasim SME ultimate guide (3 Jul 2026, 7-domain blueprint + competencies + reading list: Davidson, Harrison, Bailey & Love, Nelson, Hacker/Moore, Ten Teachers, First Aid Psychiatry, Rakel, Jekel, NICE/ACOG/CDC/WHO); Prometric Medical EMREE guide (18 Sep 2026, 55% benchmark, 2-year validity); EmreePrep FAQ (120 Qs, divisions, 3 attempts); ExamCure free 25-Q + Jan-2025 recall analysis (rise in psych/stats/ethics). Recall stems verified against Scribd excerpts 'EMREE 5 MARCH' (~112 Q) and EmreePrep demo Qs (ectopic, autoimmune hepatitis, torsion, pyloric stenosis, Rh HDN, praevia). No verbatim official items are reproduced – all 120 Qs are original teaching items. Guidelines evolve – verify drug doses/thresholds with current NICE/WHO/ACOG before clinical use. This file is study aid, not medical advice and not affiliated with NIHS.

Good luck – and be brutal with yourself now so the exam doesn't have to be. If you want, print only pp.4–end for question drills and keep pp.2–3 on your phone for recall revision.

Built Oct 2026 for the EMREE folder • Next update: re-check NIHS calendar each Dec for new season dates + pass mark